



DOCTOR AND PRESCRIBER GUIDE

# Clinical review, prescribing and care-plan oversight

A focused SecureObs guide for doctors and authorised prescribers reviewing patients, prescribing medication, checking clinical evidence and contributing to care plans.

|                                                   |
|---------------------------------------------------|
| Sign in and select ward/patient                   |
| Review patient dashboard, NEWS2 and history       |
| Prescribe regular, PRN, depot or rapid medication |
| Record allergies, ADRs and stopped medication     |
| Review notes, care plans and safety incidents     |

SecureObs

High secure ward observation workflow

3 sync loss

Medication chart

John Williams | Prescriber access

Back to observations

1 doses due soon

Yesterday and today remain open so late doses, refusals and omissions can be recorded

Admin / prescribing

Full chart

History

Patients

Room 7 | Taylor Brown

NH100421

Room 11 | Casey Black

NH100677

Room 4 | Jordan White

NH220130

Room 1 | John Doe

NH0-bm1234

Taylor Brown

Room 7 | NH100421

Allergies / ADRs

Beers

Lifeline

Save allergies / ADRs

Medication prescription

Add medicine details, start date, time prescribed, route and administration times.

Drug approved name

Dose

25/06/2026

16:53

Route

Oral

IM

Depot

S/L

Topical

Type

Regular

PRN

Depot

Rapid tranquillisation

Administration times

06:00

08:00

12:00

14:00

16:00

18:00

20:00

22:00

Additional instructions / pharmacist advice

Add prescription

Stopped medication details

25/06/2026

16:52

Reason medicine is stopped

Procydiline

250mg | Oral | Regular

Start 25 Jun 2026 16:51 | Prescribed 16:51

By John Williams

Stop medicine

| Time  | Yesterday                               | Today                                   | 26 Jun |
|-------|-----------------------------------------|-----------------------------------------|--------|
| 08:00 | <div>G Ref N X</div> <div>F S O U</div> | <div>G Ref N X</div> <div>F S O U</div> | Future |
| 14:00 | <div>G Ref N X</div> <div>F S O U</div> | <div>G Ref N X</div> <div>F S O U</div> | Future |
| 18:00 | <div>G Ref N X</div> <div>F S O U</div> | Future                                  | Future |

Lorazepam

1 mg | Oral | Regular

Start 05 Jun 2026 10:30 | Prescribed 10:30

By Dr Anita Patel

Stop medicine

Offer with evening medication round if clinically indicated.

| Time  | Yesterday                               | Today  | 26 Jun |
|-------|-----------------------------------------|--------|--------|
| 22:00 | <div>G Ref N X</div> <div>F S O U</div> | Future | Future |

Diazepam

1000mg | Depot | Depot

Start 22 Jun 2026 12:17 | Prescribed 12:17

By Dr Anita Patel

Stopped

25 Jun 2026 16:52

Discontinued by prescriber

Depot administration

Interval 14 days | Next due 09 Jul 2026 12:20

Last given 22 Jun 2026 12:20 by Dr Anita Patel

Diazepam

100mg | Depot | Depot

Start 22 Jun 2026 11:26 | Prescribed 11:26

By Dr Anita Patel

Stopped

25 Jun 2026 16:51

Discontinued by prescriber

Depot administration

Interval 7 days | Next due 29 Jun 2026 11:27

Last given 22 Jun 2026 11:27 by Dr Anita Patel

Administration and omission codes

G = Given Ref = Refused N = Route not available X = Prescriber request F = Fasting S = Self administered O = Other reason U = Product not available

## 1. What this doctor route covers

### Doctor route purpose

The doctor route is for reviewing clinical evidence, adding medical context, prescribing or stopping medication where authorised, and contributing to care-plan and safety review.

| Item                      | Guidance                                                                                                               |
|---------------------------|------------------------------------------------------------------------------------------------------------------------|
| Who should use this guide | Doctors, authorised prescribers and clinical reviewers using SecureObs.                                                |
| Prescribing access        | Doctor role can prescribe when the staff record allows prescribing. Do not prescribe under another user's session.     |
| Clinical responsibility   | SecureObs supports clinical review but does not replace professional judgement, medicines policy or urgent escalation. |

## 2. Sign in and review patient context

|          |                                                                                                                                                                                                           |
|----------|-----------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>1</b> | <b>Sign in</b><br>Use NFC staff card or STAFFCODE/PIN and confirm the correct site and ward. If using temporary access, check the allocation end time and save work when the five-minute warning appears. |
| <b>2</b> | <b>Select patient</b><br>Choose the patient and confirm name, date of birth, room and hospital or NHS number before reviewing or prescribing.                                                             |
| <b>3</b> | <b>Review dashboard/timeline</b><br>Look at latest observation location, presentation, NEWS2, food/fluid, incidents, tasks and progress trends.                                                           |
| <b>4</b> | <b>Read notes and care plans</b><br>Review recent notes and active care-plan goals before adding decisions or changing medication.                                                                        |
| <b>5</b> | <b>Check safety information</b><br>Review active safety incidents or escalation items relevant to medical review.                                                                                         |

### 3. Medication chart and prescribing

| Medication item                                                                                                                                                                                       | Doctor / prescriber guidance                                                                       |
|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------|
| <b>Allergies / ADRs</b>                                                                                                                                                                               | Review and update allergies and adverse drug reactions before prescribing.                         |
| <b>Drug approved name</b>                                                                                                                                                                             | Enter the medicine name clearly using local policy and approved naming conventions.                |
| <b>Dose and route</b>                                                                                                                                                                                 | Record dose and route such as Oral, IM, Depot, S/L or Topical.                                     |
| <b>Prescription type</b>                                                                                                                                                                              | Choose Regular, PRN, Depot or Rapid tranquillisation as appropriate.                               |
| <b>Administration times</b>                                                                                                                                                                           | Select scheduled times for regular medication. PRN/rapid items do not use routine scheduled times. |
| <b>Additional instructions</b>                                                                                                                                                                        | Add pharmacist advice, monitoring instructions or safe administration context where needed.        |
| <b>Confirmation</b>                                                                                                                                                                                   | Confirm the prescription or administration action before it becomes part of the record.            |
| <b>Stopped medication</b>                                                                                                                                                                             | Enter stop date/time and reason when discontinuing medication.                                     |
| <b>Prescribing safety</b><br>Always confirm the selected patient before prescribing. Check allergies/ADRs, active prescriptions, recent administrations and any relevant clinical risk before saving. |                                                                                                    |

Secure Obs

High secure ward observation workflow

Ready

Medication chart

SecureObs Super Admin | Administration access

Back to observations

0 doses due soon

Yesterday and today remain open so late doses, refusals and omissions can be recorded.

Admin / prescribing

Full chart

History

Patients

Room 1 | Harrison Jackson

SP-221335

Room 2 | Dean McDonald

Sp-11132

Harrison Jackson

Room 1 | SP-221335

Allergies / ADRs

Allergies

Adverse drug reactions

Save allergies / ADRs

Medication prescription

Prescription entry locked for non-prescribing staff.

Drug approved name

Dose

25/06/2026

16:43

Route

Oral

IM

Depot

S/L

Topical

Type

Regular

PRN

Depot

Rapid tranquillisation

Administration times

06:00

08:00

12:00

14:00

16:00

18:00

20:00

22:00

Additional instructions / pharmacist advice

Add prescription

Stopped medication details

25/06/2026

16:43

Reason medicine is stopped

Prosyclodean

100ml | Oral | Regular

Start 25 Jun 2026 06:40 | Prescribed 06:40

By Graham Gardiner

| Time  | Yesterday                      | Today                                                                                                                     | 26 Jun |
|-------|--------------------------------|---------------------------------------------------------------------------------------------------------------------------|--------|
| 08:00 | <div>G</div> <div>Change</div> | <div><div>G</div><div>Ref</div><div>N</div><div>X</div></div> <div><div>F</div><div>S</div><div>O</div><div>U</div></div> | Future |
| 14:00 | <div>G</div> <div>Change</div> | <div><div>G</div><div>Ref</div><div>N</div><div>X</div></div> <div><div>F</div><div>S</div><div>O</div><div>U</div></div> | Future |
| 20:00 | <div>G</div> <div>Change</div> | Future                                                                                                                    | Future |

Paracetamol

500mg | Oral | PRN

Start 25 Jun 2026 06:39 | Prescribed 06:39

By Graham Gardiner

PRN / as required

Indication/protocol: When needed, not more than 6 times per day

When needed, not more than 6 times per day

SecureObs • Doctor review, prescribing and care planning

*Medication chart: prescribing, allergies/ADRs and administration grid.*

## 4. Reviewing medication administration

- Review given, refused, omitted and unavailable doses in the chart.
- Check omission codes and notes when a dose was not given.
- Review PRN or rapid tranquillisation records in the context of behaviour, incidents and care plan.
- Use medication history to understand recent administration before changing a prescription.
- Escalate any discrepancy to the nurse in charge or medication nurse.

## 5. Care-plan and clinical record review

| Record area             | Doctor review focus                                                                                                                                  |
|-------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------|
| Patient notes           | Read recent notes before adding a medical review note. Keep entries factual, attributable and clinically relevant.                                   |
| Care plans              | Review identified needs, risks, goals, interventions and patient preferences. Add or request updates where the plan no longer matches clinical need. |
| Review date             | Check whether the care plan review date is due or clinically indicated sooner.                                                                       |
| Patient voice           | Consider what matters to the patient, patient preferences and any shared-care/family contribution that has been reviewed by staff.                   |
| Safety incidents        | Review relevant incidents, body map details, immediate action and resolution notes.                                                                  |
| NEWS2 / physical health | Use NEWS2 and observation history to support clinical assessment and escalation decisions.                                                           |
| Desktop documentation   | The authorised SecureObs staff website can be used for longer patient notes and care-plan writing. Confirm the selected patient before saving.       |

16:50

Wed 1 Jul

85

View London  
Mental Health  
NHS

Secure Obs

High secure ward observation workflow

1 savedCreate access

Patient care plans

Ash Ward | Structured nursing care planning

Back

Patients

Select a patient to create or review their care plans.

Room 1 | John Doe

Nhs-bm1234

Room 4 | Jordan White

RH220120

Room 7 | Taylor Brown

NH100421

Room 11 | Casey Black

NH100677

John Doe

Room 1 | Nhs-bm1234

1 savedCreate access

New care-plan template

Required fields are marked \*. The saved plan is signed with your staff identity.

Print current draft

Care plan title \*

For example: Emotional wellbeing and anxiety management

Identified needs / current concern \*

Describe the patient's assessed needs, current presentation and relevant context.

Risks, triggers and early warning signs

Record known risks, triggers, protective factors and signs that support may need to increase.

Goals and desired outcomes \*

What outcomes are being worked towards? Make them clear and meaningful to the patient.

Planned interventions and staff support \*

Describe what staff will do, frequency, responsibilities, escalation and how progress will be reviewed.

Patient views, preferences and involvement

Record the patient's own views, choices, communication needs and agreed preferences.

Review date \*

08/07/2026

PREPARED BY

SecureObs Super Admin (Super4dmin)

Additional notes

Optional multidisciplinary, family/carers or follow-up information.

Save signed care plan

Saved care plans

Newest plans appear first. Plans are retained as signed records.

Emotional and mental health wellbeing care plan

Created 01 Jul 2026, 16:23 by SecureObs Super Admin

Review date: 08/07/2026

Print PDFShare / save

IDENTIFIED NEEDS

Support needed when anxious and unsure of the surroundings

RISKS, TRIGGERS AND WARNING SIGNS

Loud noises and sudden movement

GOALS AND DESIRED OUTCOMES

Reassurance and discussing the situation

INTERVENTIONS AND STAFF SUPPORT

Support needed as well as some medication

PATIENT VIEWS AND PREFERENCES

John says he will let staff know when he is feeling anxious and uneasy.

ADDITIONAL NOTES

Dr to review medication before the 21/08/2026

*Care plans: structured needs, risks, goals, interventions and review date.*

## 6. Shift handover and team communication

| Area                      | Expected action                                                                                        |
|---------------------------|--------------------------------------------------------------------------------------------------------|
| <b>Handover awareness</b> | Review patient summaries where medical context affects the incoming shift.                             |
| <b>Medical action</b>     | Make clear what needs nursing follow-up, observation, physical health review or medication monitoring. |
| <b>Tasks</b>              | Create or review patient tasks where follow-up needs ownership.                                        |
| <b>Escalation</b>         | Verbally escalate urgent decisions. Do not rely solely on a written note for immediate risk.           |

## 7. What not to do

- Do not prescribe or discontinue medication under another staff member's login.
- Do not prescribe without checking allergies/ADRs and active medication records.
- Do not use SecureObs as a substitute for urgent clinical escalation.
- Do not document vague decisions such as “review later” without ownership or timeframe.
- Do not approve family/shared-care information unless it is appropriate and patient-consented according to policy.

## 8. Troubleshooting

| Issue                               | What to do                                                                                                                                                             |
|-------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------|
| <b>Cannot prescribe</b>             | Check doctor role, Can prescribe permission and medication chart module status.                                                                                        |
| <b>Patient not visible</b>          | Confirm ward selection and ask manager/admin to review patient setup.                                                                                                  |
| <b>Medication action not saving</b> | Check the named item and failure reason in sync status. Re-authenticate if requested, retry and confirm successful upload; alert the nurse in charge if time-critical. |
| <b>Wrong patient selected</b>       | Stop before saving. Return to the patient list and confirm identity.                                                                                                   |
| <b>Care-plan review unclear</b>     | Check recent notes, patient voice, incidents and handover before documenting review outcome.                                                                           |